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Management of Paxlovid Drug-drug Interactions

Drug-drug interactions can be checked at **Liverpool COVID-19 Drug-Drug Interaction website.**

If a question cannot be answered via review on Liverpool's website, the following contacts may be utilized:
0700 – 1600 Monday-Friday, except institutional holidays

Service	Contact
Non-transplant patient with pharmacist covering the care area	Page pharmacist in the care area
Non-transplant patient without pharmacist coverage	Page Antimicrobial Stewardship Pharmacist (pg#31888)
Hematopoietic stem cell transplant	Page David Frame, Denise Markstrom, or Gianni Scappaticci
Adult/pediatric kidney transplant	TC TXP PHARMACIST KP (MiChart in-basket pool)
Adult/pediatric liver transplant	TC TXP PHARMACIST LIV (MiChart in-basket pool)
Lung transplant	TC TXP PHARMACIST LUNG (MiChart in-basket pool)
Adult heart transplant	CVC HRT TXP PHARMACIST (MiChart in-basket pool)
Pediatric heart transplant	E-mail Ashley Huebschman, Bronwyn Crandall

Management of Paxlovid with calcineurin inhibitors or mTOR kinase inhibitors in solid organ or hematopoietic cell transplant

Checklist:

1) Contact transplant PharmD (Appendix B) to evaluate for **all** drug-drug interactions in table above and Liverpool reference. Avoid Paxlovid if absolute contraindications identified and holding interaction medication is not possible.
2) **Hold all calcineurin inhibitors and mTOR inhibitors at time Paxlovid is written**
3) Start Paxlovid at 24 – 48 hours from time of last dose of CNI or mTOR inhibitor (see table below)
4) Check CNI or mTOR inhibitor level per table below and restart when appropriate

Drug	When to start Paxlovid	Check level
Envarsus	**48 hours** from last Envarsus dose	Day 3 – 7 **after** last day of Paxlovid
Tacrolimus	**24 hours** from last tacrolimus dose	Day 3 – 7 **after** last day of Paxlovid
Cyclosporine	**24 hours** from last cyclosporine dose	Day 3 – 7 **after** last day of Paxlovid
Everolimus	**48 hours** from last everolimus dose	Day 3 – 7 **after** last day of Paxlovid
Sirolumus	**48 hours** from last sirolimus dose	Day 3 – 7 **after** last day of Paxlovid

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Last revised: 05/2025	**Antimicrobial Stewardship Subcommittee:** N/A	**P&T:** N/A

References

	Citation

Revision History

Date	**Revision**
01/2022	Added Appendices A and B
01/2022	Revised general recommendation
01/2022	Revised corticosteroid recommendation
02/2022	Revised general recommendation
02/2022	Updated CGRP antagonist
02/2022	Revised multiple medications
05/2022	Revised DOACs
05/2022	Added trade names
08/2022	Revised ICS, added JAK inhibitors, triptans, brincidofovir, and oxybutinin
12/2022	Removed mAb recommendations
07/2023	Revised calcium channel blockers and P2Y12 antagonist recommendations
04/2024	Added atogepant
09/2024	Added daridorexant
05/2025	Removed Michigan Medicine interaction table, updated pharmacist contacts

The recommendations in this guide are meant to serve as treatment guidelines for use at Michigan Medicine facilities. If you are an individual experiencing a medical emergency, call 911 immediately. These guidelines should not replace a provider's professional medical advice based on clinical judgment, or be used in lieu of an Infectious Diseases consultation when necessary. As a result of ongoing research, practice guidelines may from time to time change. The authors of these guidelines have made all attempts to ensure the accuracy based on current information, however, due to ongoing research, users of these guidelines are strongly encouraged to confirm the information contained within them through an independent source